PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	The views and experiences of people with acne vulgaris and healthcare professionals about treatments: systematic review and thematic synthesis of qualitative research
AUTHORS	Ip, Athena; Muller, Ingrid; Geraghty, Adam; Platt, Duncan; Little, Paul; Santer, Miriam

VERSION 1 – REVIEW

REVIEWER	Abigail McNiven University of Oxford, UK
REVIEW RETURNED	07-Aug-2020

GENERAL COMMENTS	This is the first qualitative systematic review to my knowledge of experiences of acne. The article demonstrates the strength of this approach, and covers the material comprehensively. The article is very well-written and clearly structured. The table giving representative quotes from the papers is a good addition. Would it be possible to give some demographic details when quotes are cited (if these are known) - e.g. age of study participant? In the discussion of themes, the author notes that one study (18) reported findings from a commercial trial; this could be highlighted in the section on quality appraisal (alongside any other studies in similar circumstances).

REVIEWER	Pavel Chernyshov National Medical University, Ukraine
REVIEW RETURNED	27-Aug-2020

GENERAL COMMENTS	I can't find something new in this review manuscript. I am not sure that all related publications were included. Conclusion that patients' need more information is too general.

REVIEWER	Elisabeth Svensson Sweden
REVIEW RETURNED	12-Nov-2020

GENERAL COMMENTS	bmjopen-2020-041794 The views and experiences of people with acne vulgaris and healthcare professionals about treatments: systematic review and thematic synthesis of qualitative research I have been asked to review this manuscript which, according to the title, is a systematic

	review and thematic synthesis of qualitative studies. Being a statistical and methodological expert in problem solving of studies involving qualitative data, I read the manuscript with great interest. It is a delicate task to synthesise qualitative studies scientifically and maintain the requirements of objectivity in results, of reproducibility so that another researcher could perform a similar study with the same conditions. This has not been fully taken into account. The manuscript is well-written, but there are some comments and concerns to consider. 1. Qualitative, quantitative: It is an overuse of the concepts qualitative and quantitative. These concepts are not univocal, so concepts like “quantitative research”, “quantitative papers”, qualitative research, and “qualitative papers” should be avoided, explained or specified. It would be better to describe your criteria of the qualitative study of interest for the review. As stated on page 9, some papers did dot explicitly stated “qualitative approach”. That was great! Is there a risk that this review missed some other studies because of the requirement of the concept “qualitative? 2. METHODS page 3: The meaning of the acronym ENTREC should be mentioned. 3. Search strategy: Qualitative methods, qualitative data are wide concepts, please specify or reformulate. As evident by the Table 1, different types of data seem to have been used in the studies, both quantitative (costs), dichotomous data and perhaps ordered categorical data from rating scale assessments, quality of life etc. 4. Data extraction. Reference to NVivo11? 5. Pages 4-5: Synthesis of findings: Is the thematic synthesis sufficiently detailed described so that it can be repeated? 6. Results. The results are described in text, figure 2 and in the tables 2 and 3. The use of the subheadings is appealing. - However, what about statements including “common”, “commonly”, “most participants”, and “a number of studies”? - A major concern, also mentioned in the discussion, is that eight of the 20 papers reviewed seem to involve the same patients, the same design, the same researchers, which means dependent results. Is it so that the eight papers report eight different research questions or aspects of the same project referring to the same patients, then these papers together represent one study? Then, this must be taken into account in the results section.

VERSION 1 – AUTHOR RESPONSE

	Reviewer's comments	Author's responses to reviewer's comments	Changes made to the manuscript
Reviewer 1			
1. .	The table giving representative quotes from the papers is a good addition. Would it be possible to give some demographic details when quotes are cited (if these are known) - e.g. age of study participant?	Thank you for your suggestion. I have now included demographic information (gender only) that is available from the original papers in the table of representative quotes. I did not include details such as age due to BMJ Standards on Anonymisation. A number of papers did not include demographic information and I have also included a note of this under the quality appraisal section on page 9.	Page 11 and 12, Table 3: I have included demographic information (gender) that are available for the quotes. Page 9 under quality appraisal I have included the following statement: In addition, many of the papers did not include participant characteristics when presenting quotes.
2.	In the discussion of themes, the author notes that one study (18) reported findings from a commercial trial; this could be highlighted in the section on quality appraisal (alongside any other studies in similar circumstances).	Thank you for your comment. I have now included this information in the quality appraisal section.	Page 9 added: One paper reported findings from a commercial trial which could result in bias and therefore conclusions from this study should be drawn with caution.[19]
Reviewer 2			
1.	I can't find something new in this review manuscript.	To our knowledge this is the first systematic review and synthesis of qualitative papers on acne. By synthesising the data, we present a better understanding of how people's views and experiences influence how people engage with treatment in terms of initiation and adherence. We also found	

	I am not sure that all related publications were included. Conclusion that patients' need more information is too general.	acne can have a substantial impact on people's lives and they often feel this is not recognised by others. Furthermore, people want control over their acne or acne treatment in order to alleviate psychological impact, which may include using alternative therapies or behavioural approaches such as dietary manipulation. Others carrying out qualitative research in this area will also benefit greatly from being able to draw on this overview of the literature. Please see response to reviewer 3's comment below about risk of omitting some studies due to difficulty of searching qualitative literature. While we acknowledge that good qualitative research often does produce findings that feel intuitive to clinicians, we feel that this is such an important point for the participants in these studies that it bears repeating.	
Reviewer 3			
1.	Qualitative, quantitative: It is an overuse of the concepts qualitative and quantitative. These concepts are not univocal, so concepts like "quantitative research", "quantitative papers", qualitative research, and "qualitative papers"	Thank you for highlighting this. We have added to the methods the difficulty of searching for qualitative literature and explained our definition of qualitative literature in terms of our inclusion/exclusion criteria. We have also added this as a limitation of the review.	Page 3 and 4 added: Searching for qualitative literature can be difficult and that is why we included a librarian and used the Information Specialists' Sub-Group search filter resource to ensure that all relevant terms related to acne and qualitative research were covered (see supplementary material A for the list of search terms).

	should be avoided, explained or specified. It would be better to describe your criteria of the qualitative study of interest for the review. As stated on page 9, some papers did not explicitly state "qualitative approach". That was great! Is there a risk that this review missed some other studies because of the requirement of the concept "qualitative?		We defined qualitative as papers presenting qualitative method of data collection and analysis as well as presenting qualitative data (quotes). Page 17 and 18 added: However, there is the possibility that we may have missed some studies because of our definition of qualitative and our inclusion/exclusion criteria whereby papers needed to present qualitative data, qualitative methods of data collection and analysis, and provide a sufficient amount of information about the qualitative aspect if it was part of a wider study (e.g. questionnaire development paper).
2.	METHODS page 3: The meaning of the acronym ENTREC should be mentioned.	Thank you for your comment I have spelled out the acronym ENTREQ.	Page 3 added: The Enhancing transparency in reporting the synthesis of qualitative research (ENTREQ).
3.	Search strategy: Qualitative methods, qualitative data are wide concepts, please specify or reformulate. As evident by the Table 1, different types of data seem to have been used in the studies, both quantitative (costs), dichotomous data and perhaps ordered categorical data from rating scale assessments, quality of life etc.	Thank you for your comment I agree that the term qualitative methods are a wide concept. The information about what we defined as qualitative can be found in the appendix search strategy and in our response to comment 1 above. The 'Key Themes' column relates to the themes presented in these papers. Although some terms, such as HRQL and costs can be measured quantitatively, in these papers they are considered qualitatively. We have reworded some of these for clarity.	Table 1 page 6 changed to: Impact on their quality of life Table 1 page 8 changed to: Barriers related to cost of medication and insurance coverage
4.	Data extraction. Reference to NVivo11?	Thank you for your comment I have added in a reference for NVivo 11.	Page 4 added reference to NVivo 11.

5.	Pages 4-5: Synthesis of findings: Is the thematic synthesis sufficiently detailed described so that it can be repeated?	We have described the synthesis according to the ENTREQ statement to facilitate appropriate reporting of systematic review and synthesis of qualitative papers. I have also included in this submission as per the editors request a completed PRISMA checklist.	PRISMA checklist included in submission.
6.	Results. The results are described in text, figure 2 and in the tables 2 and 3. The use of the subheadings is appealing. - However, what about statements including "common", "commonly", "most participants", and "a number of studies"? - A major concern, also mentioned in the discussion, is that eight of the 20 papers reviewed seem to involve the same patients, the same design, the same researchers, which means dependent results. Is it so that the eight papers report eight different research questions or aspects of the same project referring to the same patients, then these papers together represent one study? Then, this must be taken into account in the results section.	Thank you for your comment. We prefer to use non-numerical terms when describing the data in the results section. The number of studies reporting on each theme and how common these are, are presented in table 2. Thank you for your comment. Eight of the 20 papers were from the same author using the same sample. However, as they all focused on different research questions I have reported on these individually in the results and elaborated further on my point in the discussion.	Page 18: However, although these papers used the same sample they focused on different research questions and looked at a breadth of peoples' experiences.

VERSION 2 – REVIEW

REVIEWER	Pavel Chernyshov National Medical University, Ukraine
REVIEW RETURNED	11-Jan-2021

GENERAL COMMENTS	The title of the manuscript is: “The views and experiences of people with acne vulgaris and healthcare professionals about treatments”. However, there are a lot of data on other aspects than treatment and lack of data obtained from healthcare professionals. Therefore it is important to change the title or present data related to current title. It is unclear if authors of presented review made their conclusions based on conclusions of included articles or just on separate results from those articles (e.g. interviews of some patients or even a single patient). In present is not possible to understand from the manuscript how reliable presented conclusions are, how many acne patients or HCPs support each result, how severe acne did they have at the time of interviews. Also there was a serious heterogeneity in patients’ age (12-52 years). I would recommend to modify the structure of this manuscript according to study objectives. Each topic may have its own subheading. Under these subheadings all data should be presented in details including number of patients supporting each result or if it is not possible the number of publications presented this result and authors’ explanation on how each conclusion was made. Despite current review is based on qualitative studies it should be more “quantitative” by its nature to be interesting and valid.

REVIEWER	Elisabeth Svensson Biostatistics Sweden
REVIEW RETURNED	12-Jan-2021

GENERAL COMMENTS	The authors have considered my comments and concerns, and they are aware of the limitations.